

SYNTHETIC MEDICAL RECORD — Page 1

ASSESSMENT AND PLAN

Primary diagnosis: Atrial fibrillation with rapid ventricular response. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

CHIEF COMPLAINT

Primary diagnosis: Type 2 diabetes mellitus with hyperglycemia. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

PROGRESS NOTE

Primary diagnosis: Acute kidney injury. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

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PROGRESS NOTE

Primary diagnosis: Type 2 diabetes mellitus with hyperglycemia. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

MEDICATIONS

- Furosemide 40mg IV BID
- Vancomycin 1.25g IV q12h
- Azithromycin 500mg PO daily
- Ceftriaxone 1g IV q24h

PHYSICAL EXAMINATION

Primary diagnosis: COPD exacerbation. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

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PAST MEDICAL HISTORY

Primary diagnosis: Type 2 diabetes mellitus with hyperglycemia. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

HISTORY OF PRESENT ILLNESS

Primary diagnosis: Atrial fibrillation with rapid ventricular response. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

LABORATORY RESULTS

Potassium: 3.4 mEq/L
Glucose: 212 mg/dL
WBC: $13.4 \times 10^3/\mu\text{L}$
Creatinine: 1.9 mg/dL
BUN: 34 mg/dL

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LABORATORY RESULTS

BUN: 34 mg/dL
Glucose: 212 mg/dL
Potassium: 3.4 mEq/L
BNP: 890 pg/mL
Creatinine: 1.9 mg/dL

CHIEF COMPLAINT

Primary diagnosis: Atrial fibrillation with rapid ventricular response. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

CHIEF COMPLAINT

Primary diagnosis: Community-acquired pneumonia. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

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ASSESSMENT AND PLAN

Primary diagnosis: Cellulitis of the lower extremity. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

CHIEF COMPLAINT

Primary diagnosis: COPD exacerbation. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

LABORATORY RESULTS

BUN: 34 mg/dL
BNP: 890 pg/mL
Creatinine: 1.9 mg/dL
WBC: $13.4 \times 10^3/\mu\text{L}$
Hemoglobin: 10.8 g/dL

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MEDICATIONS

- Metformin 1000mg PO BID
- Apixaban 5mg PO BID
- Azithromycin 500mg PO daily
- Insulin glargine 20 units SC nightly

PROGRESS NOTE

Primary diagnosis: Community-acquired pneumonia. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance. Patient remains hemodynamically stable. Continues on current regimen with gradual clinical improvement noted on serial examination. Oxygen requirements decreasing. Tolerating oral intake. Ambulating with physical therapy assistance.

LABORATORY RESULTS

Potassium: 3.4 mEq/L
WBC: 13.4 $\times 10^3/\mu\text{L}$
Creatinine: 1.9 mg/dL
Glucose: 212 mg/dL
BNP: 890 pg/mL